

早稲田大学 人間科学部 英語 自習プリント

英語が得意な人のための「私大難関本番レベル」— 孤独の公衆衛生危機

2026 年 5 月 26 日

高校 3 年～既卒 (難関私大文系志望)

英語 (早稲田 人間科学部・本番水準)

Trillion 塾

第1章 (例題 — 早稲田 人間科学部レベル・5 分)

本番では 90 分で大問 4 題。本問は長文読解 1 題で 25-30 分相当。

【例題】(早稲田大学 人間科学部 入試 典型形式・改題)

Read the passage and answer the questions that follow.

For most of human history, loneliness was understood as a private misfortune — a state to be endured by individuals, perhaps eased by friends or by faith, but rarely something the state itself was expected to address. Within the last two decades, that assumption has begun to give way. Public-health officials in the United Kingdom, Japan, and several other industrialized democracies have begun to speak of loneliness not merely as an unpleasant feeling but as a measurable risk factor for early death, comparable in magnitude to long-recognized hazards such as smoking and obesity. In 2018 the British government went so far as to appoint a Minister for Loneliness; in 2021 Japan established a similar portfolio.

The medical evidence behind this shift is sober but striking. A series of large meta-analyses, drawing on hundreds of thousands of participants, have found that people who report sustained social isolation face significantly higher risks of cardiovascular disease, cognitive decline, depression, and premature mortality. The size of the effect is not small. By some estimates, chronic loneliness raises the risk of early death by approximately a quarter — a figure that would prompt urgent policy action if attached to a chemical pollutant or an infectious agent.

What makes the discovery especially uncomfortable is that the rise of reported loneliness has coincided with the period in which technological tools for connection have proliferated as never before. Citizens of the wealthiest democracies carry in their pockets devices that can place them, almost instantly, into contact with friends across the globe; and yet the same citizens, surveyed about how often they feel "left out" or "without close companions," report rising levels of distress. The simplest version of the optimistic story — that more channels of communication would mean less loneliness — has not survived the data.

Several explanations have been offered. Some researchers point to the decline of the social institutions that historically gathered people into regular contact with strangers: places of worship, trade unions, neighbourhood associations, the kinds of clubs whose

membership crossed lines of family and friendship. Others emphasize the structure of contemporary work, which increasingly delivers income through arrangements that do not require sustained co-presence with colleagues. Still others note the design of the digital platforms themselves, which often optimize for engagement — the act of returning to the platform — rather than for the slower, less measurable goods of mutual understanding.

None of these explanations excludes the others, and none should be pressed too hard. Yet they share a common implication: loneliness is not, or not only, a personal failing to be addressed by individual self-improvement. It is partly a feature of the social architecture within which individuals live, and it can therefore be altered by the deliberate redesign of that architecture. Public libraries that double as social spaces, urban planning that prioritizes the chance encounters of pedestrian streets, school curricula that explicitly teach the building and repair of friendships — none of these interventions will, by themselves, end loneliness, but each is a small recognition that the problem is not simply private.

A cautionary note is in order. The framing of loneliness as a public-health crisis runs the risk of medicalizing what is, after all, a normal part of human experience. To feel lonely after a bereavement, after a move, after the loss of a close friendship, is not to suffer from a disorder; it is to be a person to whom things have happened. The policy task is not to abolish such feelings, but to ensure that they do not, in significant numbers of cases, harden into the chronic isolation that the new research has identified as so damaging.

The deeper shift, perhaps, is in how we understand the boundary between the private and the public. If loneliness is a major determinant of health — and the evidence increasingly suggests that it is — then leaving its distribution entirely to individual fortune may be no more defensible than leaving the distribution of clean water that way. What once seemed inescapably personal has begun, slowly, to be recognized as also a matter of collective concern.

Q1. What change in attitudes toward loneliness does the first paragraph describe?

- ① The recognition that loneliness can be cured by simple medication.
- ② The shift from treating loneliness as a private misfortune to treating it as a public-health concern, including the appointment of dedicated government ministers.

- ③ The discovery that loneliness has no real effect on physical health.
- ④ The decision to abolish all government welfare programmes addressing it.

Q2. According to the second paragraph, what does the medical evidence show?

- ① That loneliness has only a negligible effect on early death.
- ② That sustained social isolation raises the risk of early death by approximately a quarter, comparable to long-recognized hazards.
- ③ That loneliness primarily affects only elderly populations.
- ④ That loneliness can be eliminated by exposure to sunlight.

Q3. Why does the author find the rise of loneliness "especially uncomfortable"?

- ① Because the most affected populations refuse medical treatment.
- ② Because it has risen during the same period that technological tools for connection have proliferated as never before.
- ③ Because the data come from unreliable surveys.
- ④ Because governments have refused to acknowledge the problem.

Q4. Which of the following is NOT cited as an explanation for the rise in loneliness?

- ① The decline of social institutions such as places of worship and trade unions.
- ② Forms of contemporary work that no longer require sustained co-presence with colleagues.
- ③ Digital platforms designed to optimize for engagement rather than mutual understanding.
- ④ A genetic mutation that has spread through the populations of wealthy democracies.

Q5. Why does the author insert a "cautionary note" about medicalizing loneliness?

- ① To argue that the public-health framing should be entirely abandoned.
- ② To distinguish normal grief and transitional loneliness from the chronic isolation that the new research identifies as damaging.
- ③ To suggest that all psychiatric diagnosis is illegitimate.
- ④ To recommend that bereaved people should never receive professional help.

Q6. Which of the following best summarizes the author's overall conclusion?

- ① Loneliness is purely a private matter and should be left entirely to individual coping.
- ② Loneliness is a major determinant of health and is now recognized, partly, as a matter of collective concern rather than purely individual fortune.

- ③ Digital platforms should be banned in order to eliminate loneliness.
- ④ The British Minister for Loneliness has already solved the problem.

孤独の概念転換 — 私的不運 → 公衆衛生問題

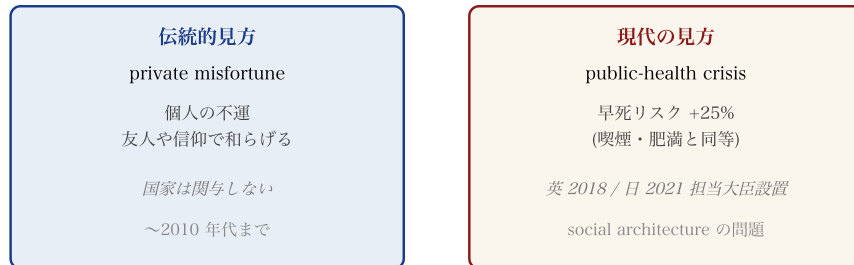


図 1-1 「私的 → 公的」の境界変容 — 人間科学が扱う典型的パラダイムシフト

第2章 (解法の全体方針と武器整理・3 分)

人科の長文は「現象提示 → 医学的根拠 → 逆説の発見 → 多角的説明 → 政策提言 → 留保 → 概念的含意」の 7 段論法。

【ジャンル分析】

本文は典型的な (A) public-health humanities essay (公衆衛生人文学エッセイ)。著者は 7 段かけて、孤独を私的なものから公的問題へと再定義する。(B) 人間科学部の長文は学際的 (interdisciplinary) テーマが頻出— 心理学・社会学・公衆衛生・都市計画が交差する論点を選ぶ。

【本問で必要な 4 つの武器】

- ① (A) データ表現の読み方 — "meta-analyses ... approximately a quarter" の統計的根拠
- ② (B) 逆説の発見 — 第 3 段の "and yet" による技術的楽観論の反証
- ③ (C) 多角的説明の併存 — "Some researchers ... Others ... Still others" の列挙構造
- ④ (D) 著者の立場 = 慎重な再定義論者と読む (medicalizing リスクへの留保を含む)

【各設問の戦略】

- Q1 (パラダイム転換) → 第 1 段 "private misfortune ... public-health concern"
- Q2 (医学的根拠) → 第 2 段 "approximately a quarter ... comparable in magnitude"
- Q3 (逆説の発見) → 第 3 段 "technological tools for connection ... and yet"
- Q4 (NOT 設問) → 第 4 段の説明リストから除外要素を消去法で
- Q5 (留保の目的) → 第 6 段 "normal part of human experience ... not a disorder"
- Q6 (全体結論) → 最終段 "matter of collective concern"

第3章 (Q1 — パラダイム転換設問・3 分)

人科 Q1 は「概念枠組みの変容」を聞く。第 1 段で即決。

【設問】 What change in attitudes toward loneliness does the first paragraph describe?

★ Step 1: 第 1 段の対比構造

- ・ 歴史的見方: "loneliness was understood as a (A) private misfortune — a state to be endured by individuals, perhaps eased by friends or by faith, but rarely something the state itself was expected to address."
- ・ 現代の見方: "Public-health officials ... have begun to speak of loneliness ... as a (B) measurable risk factor for early death, comparable in magnitude to long-recognized hazards such as smoking and obesity."
- ・ 具体例: 英 2018 / 日 2021 の Minister for Loneliness 設置

★ Step 2: paraphrase 対応

本文 "private misfortune ... public-health ... appointed a Minister for Loneliness"

選択肢② "shift from treating loneliness as a private misfortune to treating it as a public-health concern, including the appointment of dedicated government ministers"

→ 全要素対応。

★ Step 3: 誤答分析

- ・ ① "cured by simple medication" → (A) 本文に薬物治療の言及なし。
- ・ ③ "no real effect on physical health" → 本文と (B) 逆方向。
- ・ ④ "abolish all government welfare programmes" → (C) 本文と逆。 むしろ政府の関与が拡大。

【正解】 ② The shift from treating loneliness as a private misfortune to treating it as a public-health concern, including the appointment of dedicated government ministers.

第4章 (Q2 + Q3 — 医学的根拠 + 逆説・10 分)

「データ提示 → 逆説の発見」を続けて聞く。人科の中核設問配置。

【Q2 設問】 What does the medical evidence show?

★ 該当段 (第 2 段) の構造

"A series of large meta-analyses ... have found that people who report sustained social isolation face significantly higher risks of cardiovascular disease, cognitive decline, depression, and premature mortality."

具体数値: "By some estimates, chronic loneliness raises the risk of early death by (A) approximately a quarter — a figure that would prompt urgent policy action if attached to a chemical pollutant or an infectious agent."

★ Q2 の paraphrase 対応

本文 "approximately a quarter ... comparable in magnitude to long-recognized hazards"
選択肢② "raises the risk of early death by approximately a quarter, comparable to long-recognized hazards"

→ 数値・比較対象ともに直接一致。

【Q2 正解】 ② That sustained social isolation raises the risk of early death by approximately a quarter, comparable to long-recognized hazards.

【Q3 設問】 Why does the author find the rise of loneliness "especially uncomfortable"?

★ 該当段 (第 3 段) の構造

第 3 段は(A) 逆説段落:

"What makes the discovery especially uncomfortable is that (A) the rise of reported loneliness has coincided with the period in which technological tools for connection have proliferated as never before."

具体例: "Citizens of the wealthiest democracies carry in their pockets devices ... and yet the same citizens ... report rising levels of distress."

→ 「技術的接続性の爆発」と「孤独の増加」が同時に起きているという逆説。

★ Q3 の paraphrase 対応

本文 "rise of reported loneliness has coincided with ... technological tools for connection have proliferated"

選択肢② "it has risen during the same period that technological tools for connection have proliferated as never before"

→ "coincided with" → "during the same period"、"proliferated" 直接一致。

【Q3 正解】② Because it has risen during the same period that technological tools for connection have proliferated as never before.

★ 重要観察 — Q2 と Q3 の連動

Q2 = 医学的根拠の提示 → Q3 = 通念 (技術 → 接続) への反証。 (A) 「データ → 逆説」 は人科エッセイの典型 2 段ロケット。データだけで終わらず、必ず「単純な楽観論への反証」が続くのが論理の骨組み。

人科の「データ → 逆説」2 段ロケット

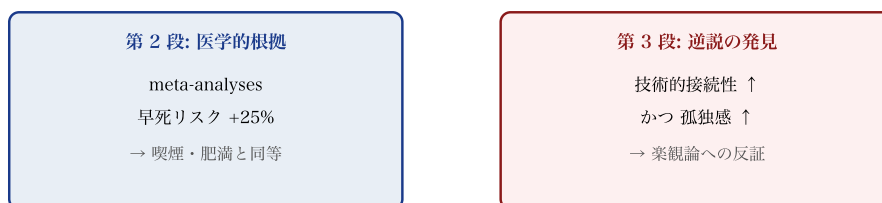


図 5-1 人間科学の核心 — データ提示後に必ず「逆説」を発見し、単純な解釈を反証する

第5章 (Q4 — NOT 設問・説明列挙・5 分)

人科頻出 NOT 設問。第 4 段の説明リストを順に消去。

【設問】 Which of the following is NOT cited as an explanation for the rise in loneliness?

★ Step 1: 第 4 段の説明を抽出

"Some researchers point to (A) the decline of the social institutions that historically gathered people into regular contact with strangers: places of worship, trade unions, neighbourhood associations"

"Others emphasize (B) the structure of contemporary work, which increasingly delivers income through arrangements that do not require sustained co-presence with colleagues"

"Still others note (C) the design of the digital platforms themselves, which often optimize for engagement"

★ Step 2: 選択肢と照合

- ・ ① "decline of social institutions" → 根拠 (A) 下線A : ○
- ・ ② "Forms of contemporary work" → 根拠 (B) 下線A : ○
- ・ ③ "Digital platforms ... optimize for engagement" → 根拠 (C) 下線A : ○
- ・ ④ "Genetic mutation" → (B) 本文に遺伝の言及なし

【正解】 ④ A genetic mutation that has spread through the populations of wealthy democracies.

💡 NOT 設問の罠は「もっともらしい科学風の説明」(遺伝・脳科学・進化心理学) を入れることが多い。本文に明示されていない限り選ばない。

第6章 (Q5 — 留保 (cautionary note) の目的・5 分)

「医療化」批判の留保段落の論理を読む。

【設問】 Why does the author insert a "cautionary note" about medicalizing loneliness?

★ Step 1: 第 6 段の構造

"A cautionary note is in order. The framing of loneliness as a public-health crisis runs the risk of (A) medicalizing what is, after all, a normal part of human experience."

具体例: "To feel lonely after a bereavement, after a move, after the loss of a close friendship, is (B) not to suffer from a disorder; it is to be a person to whom things have happened."

結論: "The policy task is not to abolish such feelings, but to ensure that they do not, in significant numbers of cases, harden into (C) the chronic isolation that the new research has identified as so damaging."

→ 留保の目的 = 「正常な悲しみ」と「慢性的孤立」を区別する。

★ Step 2: paraphrase 対応

本文 "normal part of human experience ... not to suffer from a disorder ... chronic isolation that ... damaging"

選択肢② "distinguish normal grief and transitional loneliness from the chronic isolation that the new research identifies as damaging"

→ 「正常 vs 病理」の区別が完全一致。

★ Step 3: 誤答分析

- ・ ① "entirely abandoned" → (A) 極端化。著者は public-health framing を否定していない。
- ・ ③ "all psychiatric diagnosis is illegitimate" → (B) 言い過ぎ。
- ・ ④ "bereaved people should never receive professional help" → 本文と (C) 逆方向。

【正解】 ② To distinguish normal grief and transitional loneliness from the chronic isolation that the new research identifies as damaging.

第7章 (Q6 — Main Idea 設問 + 関連話題・10 分)

人科エッセイの結論は「私的 vs 公的の境界変容」が王道。

【設問】 Which of the following best summarizes the author's overall conclusion?

★ Step 1: 最終段を精読

"The deeper shift, perhaps, is in (A) how we understand the boundary between the private and the public. If loneliness is a major determinant of health — and the evidence increasingly suggests that it is — then leaving its distribution entirely to individual fortune may be (B) no more defensible than leaving the distribution of clean water that way."

"What once seemed inescapably personal has begun, slowly, to be recognized as (C) also a matter of collective concern."

→ 主旨: ①孤独は健康の主要決定因 ②清水と同様、個人運に委ねるのは不当 ③集団的関心事として認識される必要

★ Step 2: 選択肢②との対応

"Loneliness is a major determinant of health and is now recognized, partly, as a matter of collective concern rather than purely individual fortune."

→ 3 要素すべて含む。

【Q6 正解】 ② Loneliness is a major determinant of health and is now recognized, partly, as a matter of collective concern rather than purely individual fortune.

★ 関連話題 — 人科進学後への接続

・ (A) Vivek Murthy (米国 Surgeon General) "Together" (2020) — 孤独を公衆衛生問題と公式に位置づけた医師

・ (B) John Cacioppo — Social Neuroscience の創始者、孤独と脳の実証研究

・ (C) Robert Putnam "Bowling Alone" (2000) — 社会関係資本の喪失論

・ (D) Sherry Turkle "Alone Together" (2011) — デジタル時代の孤独

・ (A) Eric Klinenberg "Palaces for the People" — social infrastructure 論

★ 大学進学後への接続

1 年次: 心理学概論・社会学概論・公衆衛生学

2-3 年次: 健康行動科学ゼミ / 環境心理学ゼミ / 老年学

卒論候補: 日本の高齢者の孤立 / SNS と若年層の孤独感 / 地域コミュニティの再設計

第8章 (人科英語 攻略のコツ・5 分)

人科長文を完答するための 5 つの技術。

【コツ 1】 7 段論法 (現象 → データ → 逆説 → 多角的説明 → 提言 → 留保 → 概念的含意) を予測

人科は学際的なので段が多い。各段の役割を 1 段 = 15 秒で把握。

【コツ 2】 統計表現の読み方

- ・ "meta-analyses" → 複数の研究を統合した最高水準のエビデンス
- ・ "approximately a quarter" → 約 25% (具体数値)
- ・ "comparable in magnitude to" → 既知のリスク (喫煙等) と比較する常套句

【コツ 3】 人科頻出語の文脈推測

determinant / risk factor / cohort / meta-analysis / chronic / transitional / social infrastructure / collective concern / medicalize / pathologize

【コツ 4】 誤答パターン 4 種 (人科版)

- ・ (A) 擬似科学: genetic mutation / brain wiring — 本文に無い「もっともらしい」説明が NOT 設問の罠
- ・ (B) 極端化: cured by medication / abolish all programmes / never
- ・ (C) 論点ずれ: 経済成長率・国際関係・宗教対立
- ・ (D) 逆方向: 著者の主張と反対

【コツ 5】 時間配分 (人科 90 分)

大問 I: 15 分 / II: 15 分 / III (本問): (A) 25-30 分 / IV: 20-25 分 / 見直し: 5-10 分

【最後にひとこと】

人科の英語は(A) 語学試験であると同時に(B) 学際的リテラシー試験。「private vs public」「individual vs structural」「normal vs pathological」の対立を頭に入れて読む。

第9章 (類題チャレンジ・10 分) [10 点]

本講義の手法を応用する 5 問。

(類題 1) 本文中の "meta-analyses" の意味として最も近いものを選びなさい。

- ① 単一の小規模な実験
- ② 複数の既存研究を統合した統計分析
- ③ 医師個人の経験的観察
- ④ 理論的推測のみで構成された論文

(類題 2) 本文の "social architecture" が示唆するものとして最も適切なものを選びなさい。

- ① 建築デザインの一様式
- ② 個人を取り巻く社会的関係や制度の構造
- ③ 国家の軍事的編成
- ④ 宗教施設の建築様式

(類題 3) 本文中で著者が言及する Minister for Loneliness の役割として最も適切なものを選びなさい。

- ① 孤独問題に取り組む政府担当大臣として 2018 年に英国で設置された
- ② 孤独を増やすことを推奨する役職
- ③ Diogenes が古代に提唱した役職
- ④ 国際連合が任命する国際担当官

(類題 4) 本文の "chronic" と対比される語として最も適切なものを選びなさい。

- ① transitional (一時的・移行の)
- ② physical (身体的)
- ③ voluntary (自発的)
- ④ imaginary (想像上の)

(類題 5) 本文の論理構成として最も近いものを選びなさい。

- ① 現象提示 → 医学的根拠 → 逆説 → 多角的説明 → 政策提言 → 医療化の留保 → 私的/
公的境界の概念的含意
- ② 結論先出し → 根拠列挙のみ
- ③ 歴史的説明のみで結論なし
- ④ 賛成意見の単純な列挙

第10章 (まとめ・宿題・5 分)

今日の核心整理と早稲田人科本番に向けた宿題

【本日の核心 5 か条】

- ① 人科エッセイは (A) 7 段論法 (現象 → データ → 逆説 → 説明 → 提言 → 留保 → 概念) が典型。
- ② (B) データ → 逆説 の 2 段ロケットを必ず捕捉 (本問 Q2-Q3)。
- ③ NOT 設問は「(C) 本文に無い擬似科学的説明」(遺伝・脳科学等) が罠。
- ④ 留保段落 (cautionary note) は「(D) 本文の主張を強化する区別」を導入する。
- ⑤ 主旨設問は(A) 「私的 vs 公的境界の再定義」を選ぶ。

【宿題】

- ① 本講義の Q1-Q6 を見ないで本文だけ読み直し、自分で 6 問の設問を作成。
- ② 類題 5 問を解き直し、本文 7 段の各段にひと言タイトルを書き出す。
- ③ 過去問: 早稲田大学 人間科学部 英語 (過去 5 年分) から長文 1 題を選び、本日の手法で精読する。

【次回授業】 人科の英作文 (200 words 程度) — "Should loneliness be treated as a public-health issue in Japan?" の論理構成

【最後にひとこと】

人科の英語は(A) 語学試験であると同時に(B) 学際的思考力試験。本問の孤独論は、入学後に学ぶ社会医学・健康社会学・環境心理学の核心テーマ。